

August 12, 2026

Molina Healthcare of South Carolina
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Kermit Trantow (MRN MUSC1075252), Claim MUSC-20251202-1F45-1

Patient	Kermit Trantow (DOB 1993-06-21)
MRN	MUSC1075252
Member ID / Group	MOL620365519 / GRP-64601
Claim number	MUSC-20251202-1F45-1
Date of service	2025-12-02
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$397.93
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-03-29

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health is submitting this formal first-level appeal on behalf of Kermit Trantow (Member ID: MOL620365519, MRN: MUSC7827100, Date of Birth: June 21, 1993) regarding the denial of Claim MUSC-20251202-1F45-1 for services rendered on December 2, 2025, by Elena V. Castellanos, MD (NPI 1881726354) at MUSC Health University Medical Center. Molina Healthcare of South Carolina denied the claim under CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") with RARC N115, citing a Local Coverage Determination, and further stated that "documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." MUSC Health respectfully disagrees with this determination and requests full reversal and payment at the allowed amount of \$289.80.

CLINICAL BACKGROUND

Mr. Trantow is a 33-year-old male with an established and documented medical history that includes active hypertension managed with Hydrochlorothiazide 25 MG, as well as a history of recurrent viral sinusitis with prior episodes recorded in 2014, 2017, and 2020. He also carries active prescriptions for Loratadine and an Epinephrine auto-injector, reflecting ongoing management of allergic and anaphylactic risk. The December 2, 2025 encounter was billed as CPT 99214, an established patient office visit of moderate complexity, with a primary diagnosis of J06.9 (Acute upper respiratory infection, unspecified) and a secondary diagnosis of R69. The visit was conducted in an on-campus outpatient hospital setting (Place of Service 22), consistent with Mr. Trantow's established care relationship at MUSC Health.

BASIS FOR APPEAL

The denial of medical necessity for this encounter is not supported by the clinical record. CPT 99214 reflects a moderate-complexity evaluation and management service for an established patient. Mr. Trantow's presentation with an acute upper respiratory infection must be considered in the context of his active comorbidities. His longstanding hypertension, currently managed pharmacologically, requires clinical consideration when evaluating and treating any acute illness, as certain common treatments for upper respiratory infections may interact with or affect blood pressure management. Additionally, his active prescriptions for an epinephrine auto-injector and loratadine indicate ongoing allergic risk that warrants careful clinical differentiation between an acute infectious process and an allergic exacerbation — a distinction that requires the level of medical decision-making consistent with a 99214 service level. The history of recurrent sinusitis further supports the clinical appropriateness of a thorough evaluation rather than a lower-acuity encounter. The payer's assertion that documentation does not establish medical necessity does not account for the complexity introduced by these concurrent active conditions. Evaluation and management of an acute upper respiratory illness in a patient with active hypertension and documented allergic risk constitutes a medically necessary service under any generally accepted standard of care. MUSC Health requests that Molina Healthcare reconsider the denial in light of the full clinical picture presented in the medical record.

REQUESTED ACTION

MUSC Health respectfully requests that Molina Healthcare of South Carolina overturn the medical necessity denial for Claim MUSC-20251202-1F45-1 and process payment at the previously determined allowed amount of \$289.80 for CPT 99214 rendered on December 2, 2025. Should Molina Healthcare require additional clinical documentation to support this determination, MUSC Health is prepared to provide the complete encounter record upon request. This appeal is submitted within the 90-day appeal window, prior to the stated deadline of March 29, 2026, and MUSC Health respectfully requests a written determination in accordance with Molina Healthcare's applicable appeal response obligations.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record